



## جوستاف روسي انترناشيونال - مصر (EPE) \* طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ليلى عبدالحليم سنهابى عزوز

المحافظة : الجيزة الحى / المدينة : بولاق الدكرور العنوان : ..

رقم البطاقة : 26708022301021 رقم التليفون : 01112849374 تاريخ تسجيل الإستمارة : 24-09-2026

العمر (تاريخ الميلاد) : 02-08-1967 المهنة : ربه منزل

### الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] قسم العلاج : الاورام

التعليق على التشخيص : .....

### تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : ..... التوقيع : .....  
الاسم : د/ندى حسن الوظيفة : ..... التوقيع : .....  
الاسم : د/نرمين رمضان الوظيفة : ..... التوقيع : .....  
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة. تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

### أعضاء اللجنة

أ.د. / ..... أ.د. / ..... أ.د. / ..... د. / ..... د. / .....

### قرار اللجنة

التشخيص النهائي : .....  
خطة العلاج : .....  
المستشفى : ..... قسم : .....  
جهة العلاج : ..... نفقات : .....  
ملاحظات : .....

ختم  
شعار الجمهورية

مدير عام  
مديرية الشؤون الصحية

رئيس  
المجلس الطبي المتخصص

|  |
|--|
|  |
|--|

|                                                                             |                                                                                     |
|-----------------------------------------------------------------------------|-------------------------------------------------------------------------------------|
| <b>Name:</b> ليلي عبدالحليم سنهال عوز<br><b>National ID:</b> 26708022301021 |  |
|-----------------------------------------------------------------------------|-------------------------------------------------------------------------------------|

|           |
|-----------|
| تقرير طبي |
|-----------|

|                  |
|------------------|
| السيد            |
| تحية طبية وبعد,, |

|                       |                       |
|-----------------------|-----------------------|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم:                 | رقم طبي:              |

|                                          |
|------------------------------------------|
| نتشرف بان نوضح فيما يلي تقريرنا عن حالته |
|------------------------------------------|

|                                             |                                             |
|---------------------------------------------|---------------------------------------------|
| تاريخ وساعة الدخول :<br>-- --:-- yyyy-mm-dd | تاريخ وساعة الخروج :<br>PM 01:17 05-07-2026 |
|---------------------------------------------|---------------------------------------------|

|                                        |
|----------------------------------------|
| وبعد الكشف والفحوصات والمتابعة وجد انه |
|----------------------------------------|

|                                                                                                     |
|-----------------------------------------------------------------------------------------------------|
| المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis. |
|-----------------------------------------------------------------------------------------------------|

|                         |                                   |
|-------------------------|-----------------------------------|
| التاريخ :<br>05-07-2026 | الوقت :<br>-- --:--               |
|                         | الطبيب المعالج :<br>د/شيماء لاشين |

[illegible]

Patient Name : - Mrs. Laila Abd Elhalim

Referred by : - Prof. Magdy Mahfouze

Date : - 06/05/26

## High resolution US and Doppler study of the left breast

- A +/- 6.8 x 4.3 cm irregular shaped heterogeneously hypoechoic solid mass lesion at the 3 o'clock meridian of the left breast with scattered calcific foci of about 1-2 mm for each, no areas of cystic changes seen.
- Doppler study revealed prominent vascularity within ,showing high RI 0.81 .
- Enlarged lymph node seen in the region of the left axilla. appears with suspicious criteria (rounded of hypoechic texture) and eccentric hilum about 3 x 1.7 cm .

### Conclusion:

- A suspicious lesion of the outer upper half of the left breast with left axillary suspicious lymphadenopathy ( BI-RAD 5 ) , for biopsy

*Thanks for your reference,*

Yours truly

DR. Ashraf Rady

Consultant Radiologist

Patient Name

: - Mrs. Laila Abd Elhalim

Referred by

: - Prof. Magdy Mahfouze

Date

: - 06/05/26

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Gustave Roussy International-Egypt- Pathology Lab  
 جوستاف روسي القتر لانتونال - مصر معمل الباثولوجي



|                 |                        |                 |                          |
|-----------------|------------------------|-----------------|--------------------------|
| Patient Name    | بيلي عبد العظيم سنهاني | Case No.        | 360291                   |
| Sex/Age         | Female 59 Y            | Patient No.     | 26295                    |
| Registered Date | 24/8/2026 03:22:42 PM  | Referred By Dr. | Interventional Radiology |
| Validation Date | 22/9/2026 01:17:35 PM  | Branch          | Gustave Roussy           |
| Reporting Date  | 22/9/2026 01:19:04 PM  | Pathology No.   | 2723/26                  |

## Pathology Report

**Clinical Data:** Left breast UOQ mass (BIRADS 5) with left axillary lymph node.

### Nature of Specimen:

- 1- Left breast mass, Tru-cut biopsy, collected on 24/08/2026 at 1:30 pm, Cold ischemia time: 20 seconds.
- 2- Left axillary lymph node, Tru-cut biopsy, collected on 24/08/2026 at 1:35 pm, Cold ischemia time: 20 seconds.

### Gross Examination:

Two containers were received, designated as follow:

- 1- Left breast mass: Fragmented grey, white tissue cores, ranging in size between 0.2 & 0.8 cm, totally submitted.
- 2- Left axillary lymph node: One grey, white tissue core, measured 1.2 cm long, totally submitted.

### Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

### Microscopic examination:

- 1- Left breast mass:
  - Histologic type: Invasive duct carcinoma.
  - Histologic grade: 2 (differentiation 2, pleomorphism 2, mitosis 2).
  - DCIS: Absent
  - Microcalcification: Absent
  - Angiolysis: Absent
  - Perineural invasion: Absent
  - Necrosis: Focal

### 2- Left axillary lymph node:

- Histopathological examination reveals fibrotic cores showing positive metastatic deposits of tumor of breast origin. No nodal tissue in examined sections.

### Results:

\* IPT (All controls showed appropriate reactivity):

- 1- Left breast mass:
  - ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة 5 - مصر  
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Miter El Qadima - Cairo, Egypt

Gustave Roussy International-Egypt- Pathology Lab  
جوستاف روسي انترنشنال - مصر - مصر لعمل الباثولوجي

|                 |                        |                 |                          |
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- PR: Moderate nuclear reaction in 1% of tumor cells (score 4/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Amplified.
- (Average Her2 copy No.: (4.6), Her2/CEP17 ratio: (2.04)).
- Ki67%: 25%.

## 2- Left axillary lymph node:

- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 10% of tumor cells (score 4/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Amplified.
- (Average Her2 copy No.: (4.05), Her2/CEP17 ratio: (2.1)).
- Ki67%: 25%.

## Conclusion:

Left breast UOQ mass with left axillary lymph node, Tru-cut biopsies:

### 1- Left breast mass:

- Invasive duct carcinoma, Grade 2.

### IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified, Group I.
- (Average Her2 copy No.: (4.6), Her2/CEP17 ratio: (2.04)).
- Ki67%: 25%.

### 2- Left axillary lymph node:

- Consistent with fibrotic cores showing positive metastatic deposits of tumor of breast origin.
- No nodal tissue.

### IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Equivocal (score 2+).
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- (Average Her2 copy No.: (4.05), Her2/CEP17 ratio: (2.1)).
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**\*\*Scoring system: "ER, PR"**

خريطة رقم (4) - الطور الأول - مبنى رقم (987) - كور نيش النيل - مصر القومية - القاهرة - مصر  
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Alsir El Qadema - Cairo, Egypt

|                 |                       |                 |                          |
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ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

**\*\* Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

*Signature*

*Dr. Walaa Ibrahim Ghonaim, MD.*



عقبة رقم (4) - القصر الأول - مبنى رقم (987) - طر شمس النيل - قصر القوية - 5 - مصر  
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Almr El Qadema - Cairo, Egypt

Patient's Name:

Leila Abdelhalim Senhabi Azzouz

Date:

2026-09-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

### Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Left breast lump.

Findings:

Right breast

- Scattered fibroglandular densities. (ACR b)
- No masses, speculations, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs. No pathological infra-clavicular nodes.

Left breast

- Scattered fibroglandular densities. (ACR b)
- Thickened left breast skin associated with underlying large infiltrative central breast mass measuring roughly (5.5 x 5.5 cm ).
- Ultrasound demonstrated large infiltrative central breast mass measuring (5.5 x 5 cm ) being more evident in the left UOQ contacting the overlying skin at 1 o'clock position .
- Thickened skin reaching 8 mm with coarsened trabecular and minimal subdermal edema.
- Infiltrated nipple.
- Multiple (more than three) pathological left axillary LNs are noted at level 1, 2 , one forming axillary soft tissue mass measuring (3.8 x 3.5 cm ).

Conclusion:-

- **Right breast (BIRADS 1): Normal study.**
- **Left breast (BIRADS 5): Left highly suspicious breast mass associated with inflammatory process and ipsilateral heavy nodal affection .**

Prof.MONA ZAKY



Patient's Name:

Leila Abdelhalim Senhabi Azzouz

Date:

2026-09-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

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Conclusion:-

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- *Left breast (BIRADS 5): Left highly suspicious breast mass associated with inflammatory process and ipsilateral heavy nodal affection .*

Prof.MONA ZAKY

## Mammography

## Ultrasound

جمهورية مصر العربية  
بطاقة تحليل الشخصية

لبنى  
عبدالحليم سنهابى عزوز  
٢١ ش دابرالناحية - الكنيسة  
بولاق الدكرور - الجيزة

١٩٧٧

٢٦٧.٠٨.٢٢٣.٠١.٢١

1S8730190

Cyst

Complex cyst

Inflammatory changes

Dilated duct

Skin changes

Nipple involvement

Chest wall

SITE

UOQ

UIQ

LOQ

LIQ

Retro-areolar

Axillary tail

N/A

Size (largest)

No mass (TO)

0-2 cm (T1)

2-5 cm (T2)

&gt;5cm (T3)

Could not be measured (Tx)

N/A

Number

Single

Multifocal

Multicentric

N/A

Axillary nodes

Non specific

Indeterminate

Pathological

N/A

BIRADS

1 - 2 - 3 - 4 - 5 - 6

Biopsy

Remarks

Biopsy

left hio  
mass  
prop

|                |                                                                                                                                                                                                    |   |
|----------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---|
| Breast Density | A                                                                                                                                                                                                  | B |
| Mammography    | <p>٢٠٢١/٠١ ٢٦٧٠٨٠٢٢٣٠١٠٢١<br/>         ربة منزل<br/>         انثى<br/>         البطاقة سارية حتى ٢٠٢٨/٠١/١٨</p>  |   |
| Ultrasound     | <p>Complex cyst</p> <p>Inflammatory changes</p> <p>Dilated duct</p> <p>Skin changes</p> <p>Nipple involvement</p> <p>Chest wall</p>                                                                |   |
| SITE           | <p>UOQ</p> <p>UIQ</p> <p>LOQ</p> <p>LIQ</p> <p>Retro-areolar</p> <p>Axillary tail</p> <p>N/A</p>                                                                                                   |   |
| Size (largest) | <p>No mass (TO)</p> <p>0-2 cm (T1)</p> <p>2-5 cm (T2)</p> <p>&gt;5cm (T3)</p> <p>Could not be measured (Tx)</p> <p>N/A</p>                                                                         |   |
| Number         | <p>Single</p> <p>Multifocal</p> <p>Multicentric</p> <p>N/A</p>                                                                                                                                     |   |
| Axillary nodes | <p>Non specific</p> <p>Indeterminate</p> <p>Pathological</p> <p>N/A</p>                                                                                                                            |   |
| BIRADS         | 1 - 2 - 3 - 4 - 5 - 6                                                                                                                                                                              |   |
| Biopsy         |                                                                                                                                                                                                    |   |
| Remarks        | <p>Biopsy</p> <p>left high</p> <p>max</p> <p>pre Ma</p>                                                                                                                                            |   |

locally  
advanced  
left  
suspicious  
mass  
(5.5 x 5 cm)

# Advanced Clinical Exam

الثدي الأيسر

الثدي الأيمن

|                                   |                                |                                   |                                |
|-----------------------------------|--------------------------------|-----------------------------------|--------------------------------|
| طبيعي                             | تراجع أو تغيير في اتجاه الحلمة | طبيعي                             | تراجع أو تغيير في اتجاه الحلمة |
| بروز الأوردة                      | تقرحات بالجلد أو الحكة         | بروز الأوردة                      | تقرحات بالجلد أو الحكة         |
| تغير في الشكل أو الحجم            | افرازات غير طبيعية             | تغير في الشكل أو الحجم            | افرازات غير طبيعية             |
| حدوث تورف                         | الشعر بأقدام بالأبط            | حدوث تورف                         | الشعر بأقدام بالأبط            |
| خفق                               | القشر بسمك عام                 | خفق                               | القشر بسمك عام                 |
| تجاويز بالجلد                     | آلم بالثدي                     | تجاويز بالجلد                     | آلم بالثدي                     |
| تحويل الجلد لما يشبه قشر البرتقال | قدم محسوس                      | تحويل الجلد لما يشبه قشر البرتقال | قدم محسوس                      |
| الإحمرار                          | تم إستصال الثدي                | الإحمرار                          | تم إستصال الثدي                |

Maximum mass dimension (mm):

Side:

Right

Left

Bilateral

Diadant:

0000

0010

0000

0010

Central and retroareolar

Axilla

Imaging study requested:

Sonography

Mammasonography

The whole breast is infiltrated (infiltratory breast) 22.

regretted case

دالة سريرية

تغير في الجلد

Patient Name

:- Mrs. Laila Abd Elhalim

Referred by

:- Prof. Magdy Mahfouze

Date

:- 06/05/26

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Pathology Report

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Nature of Specimen:

- 1- Left breast mass, Tru-cut biopsy, collected on 24/08/2026 at 1:30 pm, Cold ischemia time: 20 seconds.
- 2- Left axillary lymph node, Tru-cut biopsy, collected on 24/08/2026 at 1:35 pm, Cold ischemia time: 20 seconds.

Gross Examination:

Two containers were received, designated as follow:

- 1- Left breast mass: Fragmented grey, white tissue cores, ranging in size between 0.2 & 0.8 cm, totally submitted.
- 2- Left axillary lymph node: One grey, white tissue core, measured 1.2 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Microscopic examination:

- 1- Left breast mass:
- Histologic type: Invasive duct carcinoma.
- Histologic grade: 2 (differentiation 2, pleomorphism 2, mitosis 2).
- DCIS: Absent
- Microcalcification: Absent
- Angiolysis: Absent
- Perineural invasion: Absent
- Necrosis: Focal

2- Left axillary lymph node:

- Histopathological examination reveals fibrotic cores showing positive metastatic deposits of tumor of breast origin. No nodal tissue in examined sections.

Results:

\* IPT (All controls showed appropriate reactivity):

- 1- Left breast mass:
- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).



|                 |                        |                 |                          |
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- SISH: Amplified.
- (Average Her2 copy No.: (4.6), Her2/CEP17 ratio: (2.04)).
- Ki67%: 25%.

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- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Amplified.
- (Average Her2 copy No.: (4.05), Her2/CEP17 ratio: (2.1)).
- Ki67%: 25%.

Conclusion:

Left breast UOQ mass with left axillary lymph node, Tru-cut biopsies:

- 1- Left breast mass:
- Invasive duct carcinoma, Grade 2.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified, Group I.
- (Average Her2 copy No.: (4.6), Her2/CEP17 ratio: (2.04)).
- Ki67%: 25%.

2- Left axillary lymph node:

- Consistent with fibrotic cores showing positive metastatic deposits of tumor of breast origin.
- No nodal tissue.
- IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified, Group I.
- (Average Her2 copy No.: (4.05), Her2/CEP17 ratio: (2.1)).
- Ki67%: 25%.

\*\*Scoring system: "ER, PR"



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Alser El Qadema - Cairo, Egypt

|                 |                        |                 |                          |
|-----------------|------------------------|-----------------|--------------------------|
| Patient Name    | ليلي عبد الحليم سنهاني | Case No.        | 360291                   |
| Sex/Age         | Female 59 Y            | Patient No.     | 26295                    |
| Registered Date | 24/8/2026 03:22:42 PM  | Referred By Dr. | Interventional Radiology |
| Validation Date | 22/9/2026 01:17:35 PM  | Branch          | Gustave Roussy           |
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ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

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Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

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The ratio is calculated and the results is interpreted as:

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*Signature*

**Dr. Walaa Ibrahim Ghonaim, MD.**



عقبة رقم (4) - الدور الأول - مبنى رقم (987) - طر شيش النيل - مصر القومية - 5 مارس  
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Almr El Qadmia - Cairo, Egypt

Patient's Name:

Leila Abdelhalim Senhabi Azzouz

Date:

2026-09-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

### Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Left breast lump.

Findings:

Right breast

- Scattered fibroglandular densities. (ACR b)
- No masses, speculations, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs. No pathological infra-clavicular nodes.

Left breast

- Scattered fibroglandular densities. (ACR b)
- Thickened left breast skin associated with underlying large infiltrative central breast mass measuring roughly (5.5 x 5.5 cm ).
- Ultrasound demonstrated large infiltrative central breast mass measuring (5.5 x 5 cm ) being more evident in the left UOQ contacting the overlying skin at 1 o'clock position .
- Thickened skin reaching 8 mm with coarsened trabecular and minimal subdermal edema.
- Infiltrated nipple.
- Multiple (more than three) pathological left axillary LNs are noted at level 1, 2 , one forming axillary soft tissue mass measuring (3.8 x 3.5 cm ).

Conclusion:-

- **Right breast (BIRADS 1): Normal study.**
- **Left breast (BIRADS 5): Left highly suspicious breast mass associated with inflammatory process and ipsilateral heavy nodal affection .**

Prof.MONA ZAKY

Patient's Name:

Leila Abdelhalim Senhabi Azzouz

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Prof.MONA ZAKY

# Advanced Clinical Exam

الذي الأيسر ٢٠٢١/٩/١٢

RIS

٩٤٥٦

|                                  |                                |
|----------------------------------|--------------------------------|
| طبيعي                            | تراجع أو تغيير في اتجاه الحامة |
| بروز الأوردة                     | تقرحات بالجلد أو الحامة        |
| تغير في الشكل أو الحجم           | أقران غير طبيعية               |
| حدوث تورف                        | الشعور بأورام بالابط           |
| خفق                              | الشعور بسمك عام                |
| تجاويز بالجلد                    | ألم بالثدي                     |
| تحول الجلد لما يشبه قعر البرتقال | ورم محسوس                      |
| الإحمرار                         | تم استئصال الثدي               |

|                                  |                                |
|----------------------------------|--------------------------------|
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| الإحمرار                         | تم استئصال الثدي               |

الذي الأيمن

٩٤٥٦

Maximum mass dimension (mm):

Side:

Right

Left

Bilateral

Quadrant:

Quadrant

Quadrant

Quadrant

Quadrant

Central and retroareolar

Axilla

Imaging study requested:

Sonography

Mammosonography

OMRI breasts

The whole breast is

infiltreated (Inflammatory breast &??)

regle checked case

دالة سريرية

غير خاضعة للتأثيرات

٩٤٥٦

# Mammography

## Ultrasound

locally  
advanced  
left  
suspicious

SITE

new  
(5.5 x 5 cm)

Size (largest)

No mass (TO)

0-2 cm (T1)

2-5 cm (T2)

>5cm (T3)

Could not be measured (Tx)

N/A

Number

Single

Multifocal

Multicentric

N/A

Axillary nodes

Non specific

Indeterminate

Pathological

N/A

BIRADS

1 - 2 - 3 - 4 - 5 - 6

Biopsy

Remarks

Biopsy

left hio  
mass  
prop

جمهورية مصر العربية  
بطاقة تحليل الشخصية

لبنى  
عبدالحليم سنهابى عزوز  
٢١ ش دابرالناحية - الكنيسة  
بولاق الدكرور - الجيزة

١٩٧٧

٢٦٧٠٨٠٢٢٣٠١٠٢١

1S8730190

Cyst

Complex cyst

Inflammatory changes

Dilated duct

Skin changes

Nipple involvement

Chest wall

UOQ

UIQ

LOQ

LIQ

Retro-areolar

Axillary tail

N/A

|                |                                                                                                                                                                         |   |
|----------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---|
| Breast Density | A                                                                                                                                                                       | B |
| Mammography    | <p>٢٠٢١/٠١ ٢٦٧٠٨٠٢٢٣٠١٠٢١<br/>ربة منزل<br/>انثى<br/>البطاقة سارية حتى ٢٠٢٨/٠١/١٨</p>  |   |
| Ultrasound     | <p>Complex cyst<br/>Inflammatory changes<br/>Dilated duct<br/>Skin changes<br/>Nipple involvement<br/>Chest wall</p>                                                    |   |
| SITE           | <p>UOQ<br/>UIQ<br/>LOQ<br/>LIQ<br/>Retro-areolar<br/>Axillary tail<br/>N/A</p>                                                                                          |   |
| Size (largest) | <p>No mass (TO)<br/>0-2 cm (T1)<br/>2-5 cm (T2)<br/>&gt;5cm (T3)<br/>Could not be measured (Tx)<br/>N/A</p>                                                             |   |
| Number         | <p>Single<br/>Multifocal<br/>Multicentric<br/>N/A</p>                                                                                                                   |   |
| Axillary nodes | <p>Non specific<br/>Indeterminate<br/>Pathological<br/>N/A</p>                                                                                                          |   |
| BIRADS         | 1 - 2 - 3 - 4 - 5 - 6                                                                                                                                                   |   |
| Biopsy         |                                                                                                                                                                         |   |
| Remarks        | <p>Biopsy<br/>left high<br/>max<br/>pre-Ma</p>                                                                                                                          |   |

locally  
advanced  
left  
suspicious  
mass  
(5.5 x 5 cm)

# Advanced Clinical Exam

الثدي الأيسر

الثدي الأيمن

|                                   |                                |                                   |                                |
|-----------------------------------|--------------------------------|-----------------------------------|--------------------------------|
| طبيعي                             | تراجع أو تغيير في اتجاه الحلمة | طبيعي                             | تراجع أو تغيير في اتجاه الحلمة |
| بروز الأوردة                      | تقرحات بالجلد أو الحكة         | بروز الأوردة                      | تقرحات بالجلد أو الحكة         |
| تغير في الشكل أو الحجم            | افرازات غير طبيعية             | تغير في الشكل أو الحجم            | افرازات غير طبيعية             |
| حدوث تورف                         | الشعر بأقدام بالأبط            | حدوث تورف                         | الشعر بأقدام بالأبط            |
| خفق                               | القشر بسمك عام                 | خفق                               | القشر بسمك عام                 |
| تجاويز بالجلد                     | آلم بالثدي                     | تجاويز بالجلد                     | آلم بالثدي                     |
| تحويل الجلد لما يشبه قشر البرتقال | قدم محسوس                      | تحويل الجلد لما يشبه قشر البرتقال | قدم محسوس                      |
| الإحمرار                          | تم إستئصال الثدي               | الإحمرار                          | تم إستئصال الثدي               |

Maximum mass dimension (mm):

Side:

Right

Left

Bilateral

Diadram:

ouo

oua

olo

oia

Central and retroareolar

Axilla

Imaging study requested:

Sonography

Mammasonography

The whole breast is infiltrated (infiltrary breast) & 22.

regretted case

دالة سريرية

تغير في الجلد لما يشبه قشر البرتقال

Patient Name

:- Mrs. Laila Abd Elhalim

Referred by

:- Prof. Magdy Mahfouze

Date

:- 06/05/26

## High resolution US and Doppler study of the left breast

- o A +/- 6.8 x 4.3 cm irregular shaped heterogeneously hypoechoic solid mass lesion at the 3 o'clock meridian of the left breast with scattered calcific foci of about 1-2 mm for each, no areas of cystic changes seen.
- o Doppler study revealed prominent vascularity within ,showing high RI 0.81 .
- o Enlarged lymph node seen in the region of the left axilla. appears with suspicious criteria (rounded of hypoechoic texture) and eccentric hilum about 3 x 1.7 cm .

### Conclusion:

- A suspicious lesion of the outer upper half of the left breast with left axillary suspicious lymphadenopathy ( BI-RAD 5 ) , for biopsy

*Thanks for your reference,*

Yours truly

DR. Ashraf Rady

Consultant Radiologist



|                 |                        |                 |                          |
|-----------------|------------------------|-----------------|--------------------------|
| Patient Name    | نبلى عبد العظيم سنهالى | Case No.        | 360291                   |
| Sex/Age         | Female 59 Y            | Patient No.     | 26295                    |
| Registered Date | 24/8/2026 03:22:42 PM  | Referred By Dr. | Interventional Radiology |
| Validation Date | 22/9/2026 01:17:35 PM  | Branch          | Gustave Roussy           |
| Reporting Date  | 22/9/2026 01:19:04 PM  | Pathology No.   | 2723/26                  |

Pathology Report

Clinical Data: Left breast UOQ mass (BIRADS 5) with left axillary lymph node.

Nature of Specimen:

- 1- Left breast mass, Tru-cut biopsy, collected on 24/08/2026 at 1:30 pm, Cold ischemia time: 20 seconds.
- 2- Left axillary lymph node, Tru-cut biopsy, collected on 24/08/2026 at 1:35 pm, Cold ischemia time: 20 seconds.

Gross Examination:

Two containers were received, designated as follow:

- 1- Left breast mass: Fragmented grey, white tissue cores, ranging in size between 0.2 & 0.8 cm, totally submitted.
- 2- Left axillary lymph node: One grey, white tissue core, measured 1.2 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Microscopic examination:

- 1- Left breast mass:
- Histologic type: Invasive duct carcinoma.
- Histologic grade: 2 (differentiation 2, pleomorphism 2, mitosis 2).
- DCIS: Absent
- Microcalcification: Absent
- Angiolysis: Absent
- Perineural invasion: Absent
- Necrosis: Focal

2- Left axillary lymph node:

- Histopathological examination reveals fibrotic cores showing positive metastatic deposits of tumor of breast origin. No nodal tissue in examined sections.

Results:

\* IPT (All controls showed appropriate reactivity):

- 1- Left breast mass:
- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).



|                 |                        |                 |                          |
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- PR: Moderate nuclear reaction in 1% of tumor cells (score 4/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Amplified.
- (Average Her2 copy No.: (4.6), Her2/CEP17 ratio: (2.04)).
- Ki67%: 25%.

**2- Left axillary lymph node:**

- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 10% of tumor cells (score 4/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Amplified.
- (Average Her2 copy No.: (4.05), Her2/CEP17 ratio: (2.1)).
- Ki67%: 25%.

**Conclusion:**

Left breast UOQ mass with left axillary lymph node, Tru-cut biopsies:

- 1- Left breast mass:
- Invasive duct carcinoma, Grade 2.

**IPT:**

- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Equivocal (score 2+).
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- (Average Her2 copy No.: (4.6), Her2/CEP17 ratio: (2.04)).
- Ki67%: 25%.

**2- Left axillary lymph node:**

- Consistent with fibrotic cores showing positive metastatic deposits of tumor of breast origin.
- No nodal tissue.
- IPT:**
- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Equivocal (score 2+).
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- (Average Her2 copy No.: (4.05), Her2/CEP17 ratio: (2.1)).
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- **Left breast (BIRADS 5): Left highly suspicious breast mass associated with inflammatory process and ipsilateral heavy nodal affection .**

Prof.MONA ZAKY

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Prof.MONA ZAKY

# Advanced Clinical Exam

الذي الأيسر ٢٠٢١/٩/١٢

RIS

٩٢٥٦

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الذي الأيمن

٩٢٥٦

Maximum mass dimension (mm):

Side:

Right

Left

Bilateral

Quadrant:

Quadrant

Quadrant

Quadrant

Quadrant

Central and retroareolar

Axilla

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Mammosonography

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The whole breast is

infiltreated (Inflammatory breast &??)

regle ctel case

دالة سريرية

غير خاضعة للتأثيرات

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